

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°010/MVB_25/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (13)	ITURI : Aru, Bunia, Kilo, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU : Butembo, Goma, Oicha, Kalunguta and Katwa SOUTH KIVU : Miti-Murhesa
Reporting date	May 24, 2026
Publication date	May 25, 2026

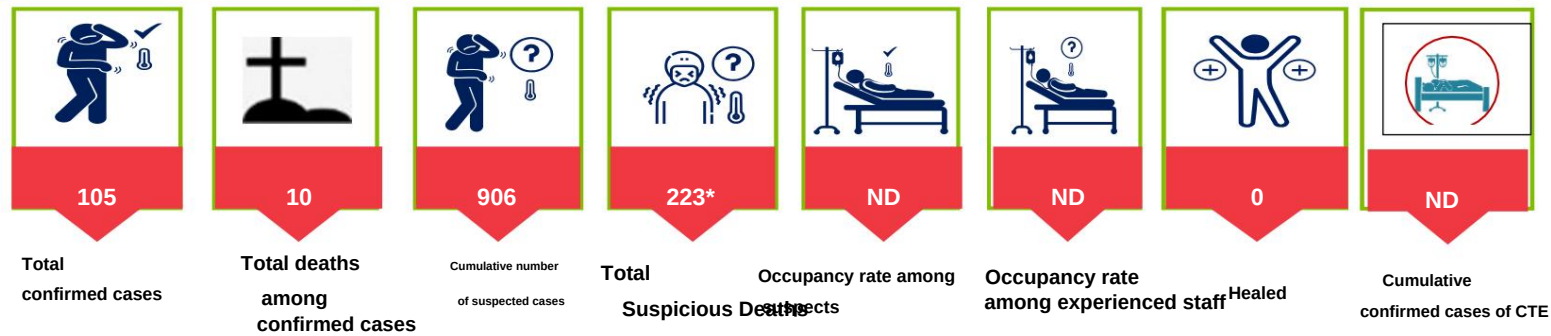


Table I. Distribution of confirmed cases and deaths by health zone in the affected provinces as of 24/05/2026

Affected provinces	Health zones with confirmed cases	New					Healed by the day
		Confirmed cases	Deaths among confirmed cases	Probable cases	Suspected cases	Listed contacts**	
Ituri	Bunia	0	0	ND	ND	0	0
	Rwampara	0	0	ND	ND	0	0
	Mongbwalu	0	0	ND	ND	0	0
	Aru	0	0	ND	ND	ND	0
	Nyankunde	0	0	ND	ND	0	0
	Nizi	0	0	ND	1	0	0
	Kilo	0	0	ND	1	ND	0
Subtotal		0	0	0	2	ND	0
North Kivu	Goma	0	0	ND	ND	ND	0
	Butembo	2	0	ND	ND	ND	0
	Katwa	0	0	ND	ND	ND	0
	Oicha***	1	0	ND	ND	ND	0
	Kalunguta***	1	0	ND	ND	ND	0
South Kivu	Miti Murhesa	0	0	ND	ND	ND	0
Subtotal		4	0	0	0	0	0
Total		4		0	2	0	0

* 0 The report of May 24 contained a data entry error.

** This data is subject to change as data reception evolves.

*** We only received the positive lab results without the other details.

I. CONTEXT

Ituri Province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction sites that attract people from other territories within the province and from neighboring provinces.

II. HISTORY OF THE EPIDEMIC

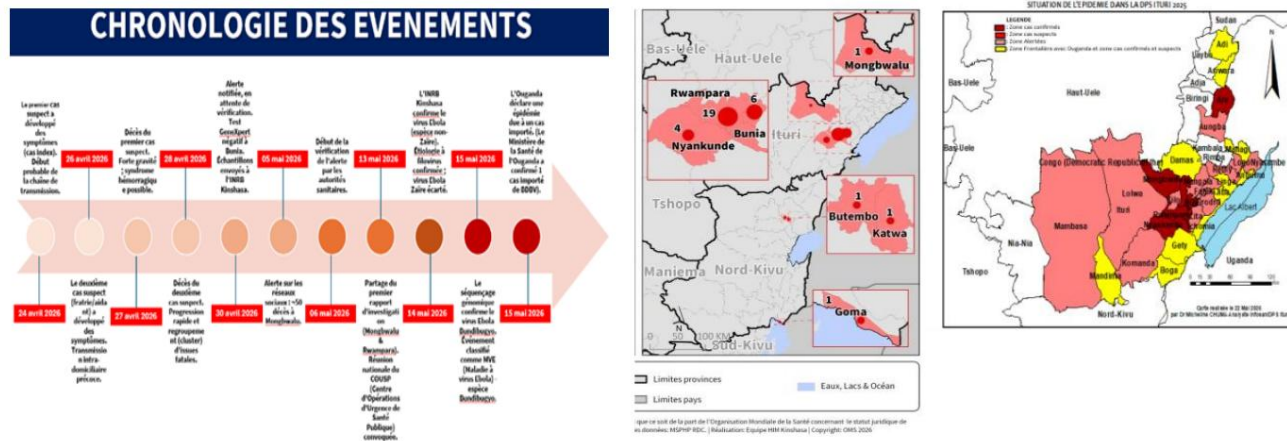
On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following the validation of these alerts, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy Rapid Intervention Teams (RITs) to Mongbwalu and Rwampara, which had reported suspected deaths of viral hemorrhagic fever among patients admitted to the Rwampara Evangelical Medical Center, who had been transferred from Mongbwalu.

Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most suspected patients or deaths presented with fever, headache, vomiting, and severe physical asthenia. Deaths occurred 2 to 3 days after the onset of illness, with symptoms including hematemesis, epistaxis, or blackish vomit. During the investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue fever, rotavirus, cholera, malaria, Yersinia pestis, measles, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

As of May 14, 2026, 13 of these 20 samples were tested, and 8 were found to be positive for non-Zairean Ebola virus disease at the INRB Kinshasa. On May 15, 2026, sequencing of the positive samples identified the BUNDIBUGYO strain, allowing the Minister of Public Health, Hygiene and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak in the Democratic Republic of Congo.

As of May 17, 2026, in consultation with member states, the WHO Director-General considers that the event taking place in DRC and Uganda meets the criteria of the definition set out in Article 1 of the IHR, and declared that this constitutes a Public Health Emergency of International Concern while specifying that the risk at the global level is low but high at the regional level.

On May 18, 2026, the Executive Director of Africa CDC officially declared a Continental Public Health Emergency (CPHE) under Article 3 of its statutes. This declaration followed an emergency meeting of the Africa CDC Advisory Group, which determined that the worsening outbreak posed an acute risk to continental health security. As a result, the Continental Incident Management Support Team was activated to provide support to affected and at-risk Member States in their response and preparedness efforts.



III. EPIDEMIOLOGICAL SITUATION UPDATE

- ÿ No samples were analyzed in Ituri.
- ÿ Affected provinces: ITURI, NORTH KIVU and SOUTH KIVU
- ÿ Cumulative confirmed cases May 24, 2026: 105 cases including 94 in Ituri, 10 in North Kivu and 1 in South Kivu
- ÿ ITURI: epicenter with 7/36 health zones affected (Aru, Bunia, Kilo, Rwampara, Mongbwalu, Nizi and Nyankunde)
- ÿ NORTH KIVU: 5/35 ZS affected (Goma, Butembo, Oicha, Kalunguta and Katwa)
- ÿ SOUTH KIVU: 1/34 ZS hit (Miti-Murhesa)
- ÿ Of the 10 confirmed deaths, 6 were male (5 over 15 years old and 1 under 15) years) and 4 females (3 over 15 years old and 1 under 15 years old)

IV. RESPONSE ACTIONS IMPLEMENTED

IV.1. COORDINATION

- Holding a strategic meeting with the Governor

IV.2. EPIDEMIOLOGICAL SURVEILLANCE

- Briefing on alert tracking at the DPS level, including members of the civil protection service (2 people) ;
- Carrying out field surveillance activities (reporting alerts, investigations, active search, listing and monitoring of contacts, retrospective documentary review of cases and deaths, etc.);

Table II. Distribution of suspected and confirmed cases, deaths of Ebola virus disease in the health zones of the Ituri, North Kivu and South Kivu DPS as of May 24, 2026

Provinces	Health zones	Number of suspected cases	Number of suspicious deaths	Number of cases Confirmed	Number of contacts*
Ituri	Aru	4	1	1	0
	Bamboo	6	2	0	183
	Bunia	249	48	24	404
	Kilo	8	2	1	0
	Mongbwalu	339	88	19	222
	Nizi	6	2	2	22
	Nyankunde	45	11	9	365
	Rwampara	228	69	32	621
	Samples without a datasheet	0	0	6	0
North Kivu	Butembo	7	0	3	22
	Goma	0	0	1	28
	Katwa	13	0	4	99
	Karissibi	0	0	0	249
	Kyondo	0	0	0	16
	Oicha**	ND	ND	1	ND
	Kalunguta**	ND	ND	1	ND
South Kivu	Miti Murhesa	1	0	1	ND
Total		906	223	105	2231

*These data are subject to change after harmonization of data at different levels of the health pyramid with the incident management system.

***: We received positive results from 4 samples without details compared to the number of samples received and analyzed for the day of 24/05/2026.

Point of Entry and Point of Control (PoE/PoC) Surveillance:

- Activations of the 6 PoCs: Airport, Dele, Mudzibala, Chai and Iga-barrier and Nizi with IOM support.

Table III. Summary of PoE/PoC data as of 24/05/2026

PoE/PoC	TOTAL
Travelers who have used the PoE/PoC	11628
Screened travelers	11143
Travelers who have washed their hands	11413
Travelers made aware of Ebola virus disease	11628
Listed contacts	0
Alerts raised	0



IV.3. LABORATORY

- 20 technicians were briefed on waste management with the support of STRIDES;

Samples	Total
Samples received	431
Re-sampling (control)	3
Total samples analyzed: 295	
Positivity rate 35.6%	
Cumulative positives 105*	

*We received positive results for 4 samples without details compared to the number received and analyzed for 24/05



IV.4. INFECTION PREVENTION & CONTROL

- Briefing of service providers from the ESS (Clinique KARIBUNI, CITADELLE, CH-UNIBU, CS Adventiste, Polyclinique Limbi Angaika & JESUS EST SAUVEUR CBCA, ... on waste management and bio-cleaning, the importance of hand hygiene, the wearing of PPE.
- Initial scorecard assessment at Clinique la RECONNAISSANCE (46%); ESS of AS Jésus est sauveur CBCA (20%), AS Bunia Cité (49%) and CH Rwankole (21%).

IV.5. HOLISTIC CARE

- Briefing of 32 service providers from Bunia General Hospital on patient care (2 doctors, 8 nurses, and 22 hygiene workers).
- Creation of a map of escapees based on lists containing their addresses.
- Start of construction work on the CTE at the HGR Bunia by the partner IMC.
- Optimization of care effective at HGR Bunia and HGR Rwampara.
- Briefing of HGR Rwampara service providers on nutritional care.
- Receipt of 50 boxes for infants under 6 months.

Table VI. Patient movement in healthcare facilities as of 24/05/2026

Indicators		HGR	SOFEPA	CH	CM	CITADEL	UNIBU	RWAKOLE	CM	BUNIA	HGR	CH	CME	RWAMPARA	HGR	HGR	HGR	HGR	Total
Report (Number of patients in bed at the end of the previous day)		10 7	2	2	2	7	2	2	8	2 44 7		2	10 19	13 33			2	2,113	
News Admissions of the day	PPL	0	3	0	0	0	0	0	0	3	2	0	0	2	0	0	0	0	5
	Others	1	0	0	2	2	0	0	0	6	1	1	2	4	5	0	0	0	15
	Total admissions of the day	1	3	0	2	2	0	0	0	9	3	1	2	6	5	0	0	0 20	
Total number of people in isolation		11 10 2		4	4	7	2	2	8	3	53 10	3	12 25	18 33			2	2,133	
Total number of patients admitted in isolation		61 13 4		8	4	7	5	2	11 3	118 38	16 NA 54	70 36	0 3 281						
Outings	Healed	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
	Deceased	0	2	0	2	0	1	0	1	0	1	7	1	0	1	2	1	0	10
	Total deaths	42 6		0	4	1	2	2	2	3	2 64	12 0			24 36	49 1		0	1,151
	No Case	0	0	0	0	0	0	0	0	0	0	0	0	0	0	2	0	0	2
	Escapees	0	3	0	1	0	0	0	0	0	4	2	0	0	2	0	0	0	6
	Transferred at HGR	0	0	0	0	0	0	0	0	0	0	2	0	2	0	0	0	0	2

IV.6. CREC

- Nearly 1,735 people (294 men, 338 women, 691 girls, 519 boys) were reached through the Red Cross volunteers during the VAD.
- Commitment from the Chief of the Bahema Irumu Chiefdom to strengthen awareness among the population of his customary entity regarding compliance with the various measures of the Ebola response. A voice message has been recorded in Swahili and the local language (Kihema) for dissemination through various communication channels.
- In parallel, the coordinator of the Civil Society Forces Vives of Ituri recorded a voice message to be disseminated through communication channels (supporting response actions by popularizing the Military Governor's communiqué on the various preventive measures of EVE including EDS, the prohibition of death wakes and the transport of lifeless bodies.
- Overcoming resistance** to EDS was broken after raising awareness among affected family members at the UNIBU Hospital Center in the Lumumba district (ZS Bunia).
- During the various services of Catholic churches and other Christian communities (Bunia and Mahagi), Ebola prevention messages were disseminated to a total of 6,700 worshippers.

IV.7. LOGISTICS

- Supply of PCI inputs to the reference ESS (CSR Lembabo, HGR Kilo, HGR Mogbwalu, HGR Lita).
- Finalization of the installation of two care tents at the Bunia General Referral Hospital (with 8 rooms and 8 bathroom)
- Work continues on installing isolation tents at Rwampara General Referral Hospital.
- Reception of 330 liters of fuel (FHI360 Support).

IV.8. SECURITY

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm
- Security briefing for newly deployed teams.
- Monitoring of the incident (isolation fire at HGR Mongbwalu).

V. CHALLENGES

- **24-hour stock shortage of RADIONE analysis kits;**
- Lack of standardized Ebola Transit and Treatment Centers in Ituri
- Lack of PCI equipment (PPE, DLM, chlorine) in North Kivu
- Low reporting of alerts by zones in the 3 affected provinces (Ituri, North Kivu and South Kivu Kivu);
- Insufficient financial resources to support all pillars of the response in the health zones affected;
- Difficulty in transporting samples to laboratories in Goma and Kinshasa.
- Spread of rumors and proliferation of traditional treatment recipes.
- Lack of communication materials and tools.
- Low diagnostic capacity
- Insufficient quantity of inputs and reagents,
- Insecurity in the health intervention zones (Rwampara CT fire, Mongbwalu).

VI. PERSPECTIVES

- Training of service providers on Ebola virus disease (EVD) and squamous cell carcinoma (SCC) surveillance in affected and high-risk health zones risk of three provinces being affected;
- Training of community health workers on Ebola virus disease and SBC surveillance in affected health zones and high risk in the province of ITURI.

FIELD PHOTOS



Work continues on the development of the Rwampara CTE



Finalization of the installation of two care tents at the Bunia General Referral Hospital (with 8 rooms and 8 toilets)



Data harmonization and status report production working session y s)

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